



PERSONA

Pass-Rate Paula

Faculty Instructor · Nursing Primary

*VRpatients Brand Foundation · Audit date 2026-05-22 · v2 · Locked palette
2026-05-25 · Run 3 of 6*

Confidence tags **[BEDROCK]** evidence-locked **[CRM-BACKED]** CRM-supported **[ASSUMPTION]** needs human input

1. Identity

Pass-Rate Paula is a community-college nursing faculty member who lives or dies by her program's NCLEX first-time pass rate, runs the simulation lab alone or with one peer, and is the single person whose ongoing case-assignment behavior decides whether VRpatients renews next July.

[BEDROCK]

2. Role Type

Primary: User + Influencer. Buyer conditional in small programs (1–4 FT faculty), where she is also the de-facto Author. **[ASSUMPTION]** (hs_buying_role 0% populated in CRM — cannot validate role-collapse frequency.)

Product touch: VR simulation (daily, where the love lives) and the authoring tool (only when forced by a missing scenario — where the pain lives). **[BEDROCK]**

3. Snapshot

- **Segment.** Community-college nursing program (ADN/AAS, or BSN-prerequisite). **[CRM-BACKED]** 248 Community College deals = 36% of typed sub-industry (N=695 of 1,228 = 57% field coverage).
- **Setting.** 1–4 FT faculty + adjuncts; one shared high-fidelity manikin; sim lab is one shared room with limited IT support. **[ASSUMPTION]** (Web inference from job postings; not CRM-measured.)
- **Tech comfort.** VR-curious, sim-pedagogy-anxious. Comfortable running pre-built cases; allergic to anything that looks like programming. **[BEDROCK]**

- **Persona priority caveat.** Paula is real but not the largest persona in the stack. **[CRM-BACKED]** Nursing/Clinical = 324 deals (27%); EMS = 780 (64%), N=1,221 typed = 99% coverage. Faculty Instructor is the academic-nursing lens — the EMS counterpart is structurally bigger.

4. Goals

- **Functional.** Get every cohort through enough varied case exposure to pass the NCLEX on first writing — without herself working more than the 45+ hours she already does. **[BEDROCK]**
- **Emotional.** Look competent in front of students and the dean. Not be the one fumbling with tech during a state BON visit. **[BEDROCK]**
- **Social.** Be the faculty member colleagues come to for simulation help. Earn an INACSL talk or a CV-line publication for promotion. **[BEDROCK]**

5. Pains (ranked, most acute first)

1. **(Functional/existential) NCLEX first-time pass rate is publicly reported and tied to state BON approval.** A single bad cohort triggers a dean's directive she cannot ignore. **[BEDROCK]**
2. **(Functional) Clinical placement sites are evaporating — pediatrics especially.** She needs simulation to fill the gap, but the platforms she has don't cover what she just lost. **[BEDROCK]**
3. **(Functional) Authoring tool learning curve costs her hours she doesn't have.** When forced into the tool because the scenario she needs isn't pre-built. **[CRM-BACKED]** Authoring-friction lost-reason cluster ~9% of typed losses (N≈25 of 276).
4. **(Financial/cultural) Flat or shrinking CC budget.** Manikin maintenance contracts (\$10K+/yr) eat the lab line; VR competes with consumables. **[ASSUMPTION]** Lit-confirmed culturally; CRM logs "Limited Budget" at only 3% of typed losses (8 of 276) — backdrop pressure, not the stated kill reason.
5. **(Emotional) Looking foolish in front of students while troubleshooting headsets, network, or audio.** Imposter syndrome around tech. **[BEDROCK]**
6. **(Functional) Documenting simulation hours and outcomes for ACEN/CCNE self-study and state BON site visits.** The paperwork takes longer than the lab itself. **[BEDROCK]**

6. What They've Tried Before & Why It Failed

Paula's lab is typically built on a high-fidelity manikin (Laerdal SimMan, CAE iStan, or Gaumard) acquired during a grant cycle, plus standalone screen-based virtual simulation (vSim, Shadow Health, Lippincott). The manikin bottlenecks 30 students through a 3-hour lab block. Screen-based vSim handles assessment-style scenarios but doesn't pressure-test clinical judgment under uncertainty the way the Next Gen NCLEX now wants. **[BEDROCK]**

Where peer VR has failed: bought during COVID emergency or peer-FOMO cycle, sat in a closet because nobody on the program had the bandwidth to author content for it, didn't renew. **[CRM-BACKED]** Pattern confirmed in CRM under "bought-but-never-used" losses (cross-referenced from Tech-Hat Tasha audit Q5); precise frequency still open.

7. Trigger Events

- **NCLEX first-time pass-rate drop** in her program — state BON publishes it; her dean reads it. **[BEDROCK]**
- **Loss of a clinical placement site** — peds, OB, or psych pulled by the hospital partner. **[BEDROCK]**
- **ACEN/CCNE re-accreditation cycle or state BON site visit** — self-study demands documented sim-based assessment. **[BEDROCK]**
- **High-fidelity manikin breakdown** or cohort expansion mandated without faculty add. **[BEDROCK]**
- **Peer faculty proof.** A peer presents a VR sim story at INACSL Annual or a state ADN meeting. **[BEDROCK]**
- **Next Gen NCLEX / NCSBN Clinical Judgment Model** language showing up in a dean directive. **[ASSUMPTION]** Strong tailwind in literature; zero CRM mentions per Tech-Hat Tasha audit — may be ahead of buying reality.

8. Objections (top 3)

1. **“You don't have pediatrics.”** — Won't work for me. **[CRM-BACKED]** Pediatrics is named on lost CC-nursing deals and pediatrics MVP (8–12yo avatars) is in active testing per Sylo. “Missing features” is 25% of typed losses (69 of 276) but undecomposed by specialty.
2. **“I'd have to learn another piece of software.”** — Won't work for me. **[CRM-BACKED]** Authoring-friction cluster ~9% of typed losses; likely larger than logged when prospect is forced into the tool to backfill missing specialties.
3. **“Will this count toward our state BON clinical-hours allowance?”** — Don't trust the vendor / wrong fit. **[BEDROCK]** Category-level objection. Rep-level documentation of state-by-state mapping converts.

9. Watering Holes

- **Associations & accreditors.** INACSL; SSH; NLN incl. NLN SIRC; Sigma Theta Tau International; AACN. Accreditors as info source: ACEN, CCNE, state Boards of Nursing. **[BEDROCK]**
- **Conferences.** IMSH (Jan, SSH flagship); INACSL Annual (June); NLN Education Summit (Sept); state NLN/ADN meetings; COADN. **[BEDROCK]**
- **Journals.** Clinical Simulation in Nursing (Elsevier/INACSL); Simulation in Healthcare (SSH); Nurse Educator; Journal of Nursing Education; Journal of Nursing Regulation (NCSBN). **[BEDROCK]**
- **Online & named voices.** HealthySimulation.com; Sim Café (Slack); SimGHOSTS. Pamela Jeffries (NLN Jeffries Sim Theory); Suzan Kardong-Edgren; Penni Watts; Beth Cusatis-Phillips; Mary “Cindy” Hallmark; Tonya Schneidereith. **[BEDROCK]**

10. Voice of Customer

Working vocabulary and forecasted phrases pulled from documented faculty terminology and adjacent VOC sources.

"Clinical judgment. Next Gen NCLEX / NGN. Case study items. INACSL Standards. 1:2 ratio. Psychological safety in debrief."

Documented faculty terminology · [BEDROCK]

"Pre-brief. Debrief. Moulage. Clinical replacement hours. Will this count toward clinical hours under our state board? Are our graduates 'Nurse Ready'?"

Documented faculty terminology · [BEDROCK]

Plausible 11pm Google search: *"pediatric simulation scenarios free download" / "Next Gen NCLEX case study examples free."* **[ASSUMPTION]** Reconstructed from documented faculty terminology; not pulled from real search-volume data. Routed to P7 / Tech-Hat Tasha Q16 for verbatim verification.

11. Before / After

BEFORE VRpatients works

- Patches her curriculum with whatever the dean approved last grant cycle.
- The peds rotation she counted on just got pulled by the hospital partner; she's googling free peds scenarios at 11pm.
- Dreads opening the authoring tool. Her last NCLEX cohort underperformed and the dean wants a clinical-judgment story by next month.
- Half-knows her sim hours might not pass the next BON visit.

AFTER VRpatients works

- Runs validated, NGN-aligned cases from a library that covers the placements she lost.
- Has an INACSL/NCJMM crosswalk she drops into her ACEN self-study.
- Debriefs feel competent in front of students. The dean asks her to present at the state ADN meeting because the program's sim hours look defensible.
- Program just hit the 90% pass-rate state BON threshold. **[ASSUMPTION]** NGN-library + crosswalk contingent on product / messaging work not yet shipped.

12. Confidence Ledger

[BEDROCK]

- NCLEX existential pressure.
- Clinical-placement loss as trigger.
- INACSL/SSH/NLN watering holes.
- Documented faculty workload + burnout.

[CRM-BACKED]

- EMS dwarfs Nursing (780 vs 324; N=1,221 = 99% coverage).
- Community College sub-industry ≈ 248/695 typed (36%).
- Authoring-friction lost-reason cluster ~9% of typed losses (N≈25/276).

[ASSUMPTION]

- Role-collapse frequency (hs_buying_role 0% populated).
- NCLEX/NGN as actual call-language (0 CRM mentions).
- VOC synthesized from literature.

- Pre-built-case love and authoring-tool hate as a pattern in the literature.
- Pediatrics MVP in active testing per Sylo.
- Financial-budget pressure narrative (CRM-logged at only 3% of typed losses).
- Whether peds is the deal-killer or stand-in for deeper objection.

Open Questions Routed to Team

Full traceable list in brand/personas/persona-audit-open-questions.md → “Pass-Rate Paula” section. Summary by team:

- **Sales + CS.** Role-collapse in CC programs (P1); Does Paula's usage predict renewal? (P5).
- **CS + Product + Sales.** “Scenario building” — customer- or VRp-built? (P2).
- **Sales + Marketing.** NCLEX/NGN language on Paula calls? (P3); INACSL/SSH/NLN — actual deal-origination channels (P7).
- **Sales.** Who signs the PO — Paula, sim coord, director, dean? (P4); Pediatrics: named vs. deal-killing objection (P6).
- **Marketing + CS.** Faculty Champion program viability (P8).

Composite persona. No single named customer was used. Aggregate patterns only. Pre-questionnaire base draft — not for promotion to outputs/final-deliverables/ until the sales / CS / product / leadership questionnaire closes the [ASSUMPTION] gaps.